

Sante Speech Health Summary

Report ID: sum_99fb9a19e184 | Generated: 2026-02-15 20:25 UTC

Session: conversation | Duration: 0.0s

Executive Summary

Data quality is limited; treat outputs as coarse exploratory estimates only.

- Intoxicated Speech Pattern Signal: exploratory-low (score 10)
- Sick / Tired Voice Pattern Signal: exploratory-low (score 8)
- Aphasia-like Language Pattern Flag: exploratory-low (score 8)

Estimates

Intoxicated Speech Pattern Signal | exploratory-low | score 10

Suggestion: Low signal in this sample. Monitor trend across repeated sessions and retest with a matched microphone/device/codec setup before drawing conclusions.

Evidence: Short-pause index 0.11; Slow-rate index 1.00; Loudness mean 0.276; Jitter 0.037

Limitations: Sleep deprivation, stress, and microphone clipping can mimic this pattern.; Exploratory speech biomarkers are probabilistic, not diagnostic outcomes.

Sick / Tired Voice Pattern Signal | exploratory-low | score 8

Suggestion: Low signal in this sample. Monitor trend across repeated sessions and retest with a matched microphone/device/codec setup before drawing conclusions.

Evidence: Long-pause index 0.00; Energy-drop index 0.76; Low-loudness index 0.84; HNR 7.20 dB

Limitations: Cannot separate respiratory causes from fatigue without clinical context.; Exploratory speech biomarkers are probabilistic, not diagnostic outcomes.

Stress Activation Speech Signal | exploratory-low | score 5

Suggestion: Low signal in this sample. Monitor trend across repeated sessions and retest with a matched microphone/device/codec setup before drawing conclusions.

Evidence: Speech-rate proxy 2.00 peaks/s; Short-pause index 0.11; Pressured-tempo index 0.36; Fast-rate index 0.00

Limitations: High arousal can also reflect urgency, excitement, or speaking style.; Exploratory speech biomarkers are probabilistic, not diagnostic outcomes.

Aphasia-like Language Pattern Flag | exploratory-low | score 8

Suggestion: Low signal in this sample. Monitor trend across repeated sessions and retest with a matched microphone/device/codec setup before drawing conclusions.

Evidence: Phoneme coverage 33 tokens; Flagged disfluencies 30; Voiced segment rate 2.44/s; Lexical diversity 1.00

Limitations: Cannot subtype aphasia from this short speech sample.; Exploratory speech biomarkers are probabilistic, not diagnostic outcomes.

Transcript Snapshot

Thank you. Bye-bye.

Research-only estimate report. It is not a medical diagnosis. Use in combination with formal clinical assessment and environmental quality checks.